

The Burn Center of the Caribbean

Feasibility Brief — Public Summary

Feasibility-stage summary for prospective partners and supporters. Figures are directional pending primary-data validation. Not legal, tax, clinical, or investment advice.

1. The Opportunity

Puerto Rico — an island of roughly 3.2 million residents — has no centralized burn center of its own. The most serious cases are delayed and frequently flown more than a thousand miles to the mainland, away from families during the most frightening days of a child's life, and at catastrophic cost. We are developing the island's first comprehensive, child-focused burn center — paired with a financial safety net so that no family is bankrupted for saving their child's life.

2. Mission, Vision & Pillars

Mission. The Burn Center of the Caribbean protects the children of Puerto Rico from the lasting harm of severe burns — healing their injuries close to home, and shielding their families from the catastrophic medical debt that can turn one tragic moment into lasting poverty. Every child is treated with dignity and expertise, regardless of what their family can pay.

Misión. El Centro de Quemados del Caribe protege a la niñez de Puerto Rico del daño duradero de las quemaduras graves: sanando sus heridas cerca de casa y protegiendo a sus familias de la deuda médica catastrófica.

Vision. A Caribbean where every child can heal from a burn close to home, and no family is bankrupted for saving their child's life.

Four pillars:

- Charity care — free or subsidized burn treatment for children of low-income families.
- Family financial protection — a fund that absorbs catastrophic burn costs and heads off the medical debt that drives household impoverishment.
- Prevention where risk is highest — burn-safety outreach in underserved communities.
- Recovery & reintegration — rehabilitation and scar care that keep an injured child from being disabled out of school and future earning power.

3. The Need

- No centralized burn facility serves Puerto Rico's ~3.2M residents.
- Only four professionals on-island are certified in Advanced Burn Life Support (ABLS) — an emergency burn-care course open to nurses, EMS, ER physicians and surgeons, not a surgical credential — alongside a documented shortage of fellowship-trained burn surgeons (JBCR, 2025).
- US guideline benchmark: at least one burn center per million people, and 1–2 burn surgeons per 500k–1M; Puerto Rico falls far below.
- Documented delays average roughly six days to reach a burn surgeon, with most patients passing through at least one other facility first.

- Severe cases are transferred to the mainland — e.g., the widely reported Puerto Rican patient billed \$1.7M at Brooke Army Medical Center — illustrating both the dependence and the human cost.
- Puerto Rico's high diabetes prevalence (~40%) drives a large chronic-wound population, expanding the clinical need beyond acute burns.

4. The Workforce Reality

Burn surgery is among the scarcest specialties in American medicine — on the order of ~300 burn surgeons nationally, clustered in mainland academic centers, in a field that is aging and contracting. Puerto Rico's absence of a burn surgeon reflects this uneven national distribution. Our approach does not wait for an impossible number of specialists: we recruit one expert burn surgeon and multiply their reach through a bench of burn-trained nurse practitioners.

5. The Care Model — Two-Tier Hub-and-Spoke

Tier 1 — the hub (surgeon-anchored). A central burn center for inpatient and critical care, surgical excision and grafting, emergency stabilization, and complex case management — reinforced by telemedicine backup, a mainland verified-center affiliation, transfer protocols, and, as the program matures, a second surgeon.

Tier 2 — the spokes (NP-led). Outpatient burn and wound-care clinics at multiple sites across the island, staffed by nurse practitioners who provide daily continuity: wound assessment and dressing changes, infection and healing monitoring, scar management, follow-up, family education, and triage and escalation. This is where most patient volume and the island's geographic-access gap live.

Connective tissue. Telemedicine links the surgeon to the spokes between in-person days, and clear escalation and transfer criteria (aligned to American Burn Association referral criteria) route patients to the hub when needed. The model extends the scarce resource without diluting it. Practice details under Puerto Rico's nurse-practitioner framework will be confirmed with the PR Board of Nursing before scaling.

6. Funding Approach

The Center's charitable arm is being established as a Puerto Rico nonprofit, intended to qualify — once certified and listed by the relevant authorities (CECFL / DDEC) — as a recipient of Act 60 child-poverty donations, and to draw on a diversified funding base: disaster-preparedness funding, disease-specific grants and philanthropy, regional self-pay, and clinical reimbursement. The foundation will be governed independently of its donors, with restricted-funds accounting that keeps charitable dollars funding charitable care. Charitable listing and tax-exempt status are pending.

7. Regulatory & Accreditation Pathway

- Clinical/hospital: CMS conditions of participation, EMTALA, HIPAA, territory licensing, and hospital accreditation (Joint Commission).
- Burn credibility: American Burn Association Burn Center Verification across the full continuum of care is the standard we are building toward.

- Nurse-practitioner model: collaborative-agreement terms, supervision ratios, and prescriptive authority confirmed with the PR Board of Nursing.
- Nonprofit/funding: nonprofit formation, PR tax-exempt status, and child-poverty certification, with restricted-funds accounting.
- Professional counsel — healthcare, PR nonprofit/tax, and nursing-regulatory — is engaged to shape structure and certification before commitments.

8. Key Considerations & Risks

We hold these openly with partners:

- Recruiting scarce specialist talent — materially mitigated by the nurse-practitioner multiplier model, with reliance on a single surgeon reduced as a second is added.
- A challenging reimbursement environment and Medicaid funding uncertainty — addressed by diversifying revenue beyond clinical reimbursement.
- Certification is plausible but not automatic — being pursued directly with the relevant Puerto Rico bodies.
- Nurse-practitioner scaling depends on regulatory ratios and sound supervision and liability protocols.
- Capital intensity of the inpatient hub — de-risked by affiliating with or embedding in an existing Puerto Rico hospital rather than building greenfield.
- Coordination across a hybrid clinical model — addressed with shared protocols and order sets from day one.

9. Ecosystem & Partners

- American Burn Association — verification, ABLS training pathway, and workforce data.
- Phoenix Society for Burn Survivors — survivor support and advocacy.
- Puerto Rico institutions — the PR Medical Center trauma complex and the VA Caribbean.
- Aligned Puerto Rico nonprofits supporting burn-injured children.
- Puerto Rico medical schools — toward a long-term burn fellowship pipeline for the wider Caribbean.

10. Status & Next Steps

This initiative is at the feasibility stage. Immediate next steps:

- Finalize the mission and one-page case for support.
- Establish the nonprofit arm and begin tax-exempt and child-poverty certification.
- Confirm the nurse-practitioner framework with the PR Board of Nursing.
- Determine site and structure — a host-hospital embed versus greenfield, and a mainland affiliate.
- Size demand and model two-tier unit economics to break-even.
- Build the talent plan — surgeon recruitment and an NP upskilling pipeline.
- Assemble the funding stack — donor outreach plus disaster-preparedness and grant targets.

Feasibility-stage public summary. Figures are directional pending primary-data validation. The Center's charitable arm is being established; charitable listing and tax-exempt status are pending. Confirm all legal, tax, nursing-regulatory, and certification specifics with appropriate Puerto Rico counsel before any commitment. A detailed working brief is shared with partners and funders on request.